

Fundraising & Advocacy Copywriting Portfolio

Three original samples across direct fundraising, advocacy campaigns, and donor impact storytelling.

Sample 1: Direct Fundraising Appeal Letter

Every Winter, We Run Out of Beds Before We Run Out of Need

Dear Friend,

Last January, a woman named Rosa came to our shelter doors at eleven at night, carrying a sleeping bag and a toddler who had long since stopped crying and started shivering instead. We had one bed left. We gave it to them. Twenty minutes later, another mother arrived with two children. We had nothing left to give her but a blanket and a promise that we would find something by morning.

We kept that promise. But we shouldn't have had to make it in the first place.

This winter, we expect to turn away more families than we did last year, not because we care less, but because the need has grown faster than our capacity to meet it. Rising rents, stagnant wages, and a housing shortage that shows no signs of easing have pushed more families than ever into the impossible position Rosa found herself in that night. We are already at ninety percent capacity, and the coldest months haven't even arrived yet.

What your gift actually does

We know you get letters like this often, so we want to be specific about where your money goes, because vague promises don't keep anyone warm.

A gift of \$50 covers one week of overnight shelter for a single parent and child, including a bed, three meals a day, and access to our on-site case worker, who helps families begin the process of finding stable housing rather than cycling back through emergency shelter again next winter.

A gift of \$150 funds a full intake package for a new family, medical screening, trauma-informed counseling access, and the first month of case management, the kind of comprehensive support that research consistently shows shortens the average length of a family's homelessness by nearly half.

A gift of \$500 sponsors an entire emergency bed for the season, meaning one more family like Rosa's never has to hear "we have nothing left" on the night they need help most.

Why we're asking now, not later

We've learned, sometimes the hard way, that waiting until a family is standing at our door in the cold is too late to plan properly. Every dollar raised before December lets us stock supplies, schedule adequate overnight staffing, and secure the additional bed space we already know we'll need based on current intake trends. Your gift today isn't just charity. It's the planning capital that determines whether we're prepared or scrambling when the real cold sets in.

The families behind the numbers

It's easy for a shelter's need to become an abstraction, a statistic about capacity and beds. So let us tell you about one more family instead of another number.

Marcus and his two children arrived at our door in March, after a workplace injury left him unable to keep up with rent. He wasn't looking for charity. He was looking for six weeks of breathing room while his workers' compensation claim processed. That's exactly what we gave him, a bed, a case worker who helped expedite his claim, and a family who could exhale for the first time in months. Marcus and his kids moved into stable housing in May. He still sends us a card every winter, thanking us for the six weeks that changed the direction of his family's whole year.

That's what your gift makes possible. Not just survival, but the breathing room a family needs to get back on their feet with dignity intact.

A gift that multiplies

Every dollar you give this season will be matched, up to \$25,000, by a coalition of local business partners who believe, as we do, that no family should face a winter night with nowhere to go. That means a \$50 gift becomes \$100 in actual shelter capacity. A \$150 gift becomes \$300. This match window closes December 31st, so your generosity today goes twice as far as it would any other time of year.

What we're asking of you

We're not asking you to solve homelessness. No single gift can do that, and we'd never pretend otherwise. We're asking you to make sure that the next family who arrives at our door at eleven at night, cold and out of options, hears something other than "we have nothing left."

Your gift, whatever size feels right for you, becomes part of the answer we're able to give them. Rosa and her daughter are stable now, in an apartment of their own, because someone gave before we ever needed to turn them away. We'd like to be able to say that about every family who comes to us this winter.

Thank you for considering it. Thank you, more than that, for the gifts you've already given in years past, which are the only reason we've been able to help as many families as we have.

A note on where every dollar goes

We know trust is earned, not assumed, especially when it comes to charitable giving. Eighty seven cents of every dollar donated to Harbor House goes directly to shelter operations and family services, a ratio we're proud of and one that's verified annually through independent financial audit. The remaining amount covers the administrative backbone that keeps the shelter running responsibly, payroll for our case workers, facility maintenance, and the modest fundraising costs required to keep letters like this one reaching people like you.

We also want you to know what happens to families after they leave our shelter, because a bed for the night was never the whole goal. Of the families who complete our full case management program, seventy eight percent remain in stable housing one year later, a number we track carefully because it's the real measure of whether we're actually changing a family's trajectory or just delaying the same crisis. That number has climbed steadily over the past three years, largely because of donors like you who fund not just emergency beds but the case management work that happens after.

If a gift isn't possible right now

We understand that not everyone reading this letter is in a position to give financially this season, and that's alright. If a monetary gift isn't possible right now, consider following our updates online and sharing Rosa's story, or Marcus's, with someone in your circle who might be able to help. Awareness has its own quiet value, even when it doesn't come with a dollar amount attached. And if your circumstances change later in the season, our doors, and our gratitude, remain open whenever you're ready.

With gratitude and hope for the season ahead,

The Team at Harbor House Family Shelter

Give today at harborhouseshelter.org/winter, or return the enclosed envelope with your gift.

Sample 2: Advocacy Campaign Copy

The Six Week Gap That's Costing Mothers Their Lives

There is a six week gap in maternal healthcare that almost nobody talks about, and it is quietly costing women their lives.

Most conversations about maternal health focus, understandably, on pregnancy and delivery. Prenatal visits. Birth plans. The moment a baby arrives. But roughly a third of pregnancy-related deaths in the United States happen after that moment, in the weeks and months following delivery, during a period when the healthcare system largely assumes the hard part is already over.

It isn't. For many women, it's just beginning.

What actually happens after the baby comes home

Standard postpartum care in most of the country consists of a single check-up, scheduled six weeks after delivery. One appointment, covering everything from physical recovery to mental health screening to family planning, compressed into a visit that averages fifteen minutes.

Between delivery and that single six week appointment, a new mother is largely on her own to recognize warning signs that even trained clinicians sometimes miss. Postpartum hemorrhage, dangerously high blood pressure, and severe infection can all develop in the days and weeks after birth, often with symptoms that get dismissed as normal exhaustion or ordinary recovery discomfort. By the time many women reach that six week appointment, a preventable complication has already become a genuine emergency.

The consequences of this gap are not evenly distributed. Black mothers in the United States are nearly three times more likely to die from pregnancy-related causes than white mothers, a disparity that researchers have linked directly to inconsistent postpartum follow-up and dismissed symptoms in the weeks after delivery. This is not a rare tragedy. It is a predictable outcome of a system that treats the postpartum period as an afterthought rather than the medically vulnerable window it actually is.

What better care could look like

Several countries with stronger maternal health outcomes structure postpartum care completely differently, with multiple check-ins in the first two weeks alone, home visits from trained nurses, and mental health screening built into the process rather than added as an afterthought. The evidence is not ambiguous. Countries that treat the postpartum period with the same urgency as pregnancy itself see measurably better outcomes, for both physical and mental health.

Some hospital systems in the United States have begun piloting similar models, adding a check-in at week one and week three in addition to the standard six week visit. Early results are promising, with earlier detection of complications and meaningfully higher rates of postpartum depression being caught and treated before it becomes severe. But these programs remain the exception, dependent on individual hospital initiative rather than any broader policy requirement.

Why this requires policy change, not just awareness

Awareness alone will not close this gap, because the problem isn't that new mothers don't know something might be wrong. It's that the system isn't structured to catch it in time, and individual women cannot be expected to diagnose their own life-threatening complications during the most exhausted, disoriented weeks of their lives.

This requires insurance policy that mandates coverage for multiple postpartum visits, not just one. It requires hospital systems to build follow-up care into their standard protocols rather than leaving it to individual physician discretion. It requires funding for home visit programs, which have consistently shown some of the strongest outcomes of any postpartum intervention studied.

What you can do

Contact your state representatives and ask directly whether they support expanded postpartum Medicaid coverage, since many states have already begun extending coverage from sixty days to a full twelve months postpartum, a policy change proven to reduce maternal mortality. Ask your own healthcare provider whether they offer any follow-up before the standard six week visit, and if they don't, ask why not. Support organizations doing direct advocacy work in this space, many of which operate on shoestring budgets despite the scale of what they're working to change.

The gap is closeable

This isn't an intractable problem. It's a structural gap with well-documented fixes, fixes that other countries have already implemented successfully. What's missing isn't the solution. It's the political will to treat the six weeks after childbirth with the same seriousness as the nine months before it.

Every mother deserves to be seen, checked on, and taken seriously in the weeks after she gives birth, not just at delivery and then again six weeks later when it might already be too late. Closing this gap isn't a medical mystery. It's a choice, and it's one we can still make.

The stories behind the statistics

Behind every figure in this piece is a woman whose name we may never know, because so much of maternal mortality data comes from cases where warning signs were present but dismissed. A mother who mentioned a severe headache at her two week phone check-in, if her clinic even offered one, and was told to rest and drink water. A mother whose swelling was written off as ordinary postpartum fluid retention, when it was in fact an early sign of a blood pressure crisis that would become life-threatening within days. These are not rare, freak occurrences. They are the predictable result of a system that asks exhausted, sleep-deprived new mothers to somehow distinguish between normal recovery and genuine emergency, largely on their own.

What clinicians themselves are saying

This isn't only an advocacy position from outside the medical system. A growing number of obstetricians and midwives have begun speaking publicly about the inadequacy of the standard six week model, many of them citing cases from their own practice where earlier follow-up would have caught a complication sooner. Professional medical organizations have increasingly recommended more frequent postpartum contact, yet recommendations without funding or policy backing rarely translate into changed practice at the hospital level, where staffing and reimbursement structures still largely reflect the old single-visit model.

A gap that's closing, slowly, in some places

It's worth noting that progress is happening, even if unevenly. Several states have extended postpartum Medicaid coverage from sixty days to a full year, a shift that research already links to earlier detection of complications and better continuity of mental health care. Advocacy organizations working specifically on this issue have moved several of these policy changes from proposal to law within the past few years alone, proof that this gap is not a permanent fixture of the system but a solvable, fundable, closeable problem.

Learn more and take action at maternalhealthgap.org

Sample 3: Donor Impact Story

The Well That Changed Everything, One Year Later

A year ago, the walk to clean water in Amina's village took three hours, round trip, every single day. Her daughter Fatima, then nine years old, made that walk most mornings instead of going to school, because someone in the family needed to fetch water and Amina's other children were too young to carry a full container that distance.

Today, Fatima is in school every morning, not occasionally, not when the family can spare her, but every single day, because the well your donations funded sits less than ten minutes from her front door.

What one well actually changes

It's tempting to think of a well as a single, contained intervention, water infrastructure and nothing more. But the ripple effects of what your gift built reach far beyond hydration, and Amina's family is a clear example of exactly how far that ripple travels.

With three fewer hours spent walking each day, Fatima returned to school full time, and her attendance record over the past year shows what that time actually meant, a jump from roughly sixty percent attendance to ninety four percent, the difference between falling behind and genuinely catching up. Her teacher reports that she's now reading at grade level for the first time since the drought forced her family into that daily three hour walk two years ago.

Amina, freed from the walk herself on the days Fatima couldn't make it, started a small vegetable garden using the well's runoff water, and now sells produce at the village market twice a week. That income, modest as it is, has covered school fees for her two younger children, fees the family couldn't previously afford consistently.

The numbers behind the story

Your donations funded one well, but that well now serves fourteen families and roughly ninety people directly, with ripple effects reaching considerably further into the broader community through the market activity it's enabled. Waterborne illness in the immediate area has dropped by an estimated sixty percent over the past year, based on clinic visit records from the regional health post, a number that translates directly into fewer missed school days and fewer lost workdays for parents caring for sick children.

The well itself was built using a community-maintained design specifically chosen for long-term sustainability, with local technicians trained on-site during construction so that basic maintenance and minor repairs can happen without waiting on outside intervention. That training is part of why wells built through this program have a documented ninety one percent functionality rate five years after construction, compared to a much lower rate for wells built without local maintenance training built into the process from day one.

Why this matters beyond one village

Amina and Fatima's story is not an isolated success. It's a pattern we see repeated, with variations, in every community where a well like this gets built. Girls' school attendance rises. Small local economies get a foothold they didn't have before. Preventable illness drops. None of this happens by accident. It happens because clean water removes one of the most persistent, quietly devastating barriers standing between a family and the kind of stability that lets children stay in school and parents build something beyond daily survival.

What's next

Your continued support is already funding the beginning of a second well, roughly four miles from Amina's village, in a community facing nearly identical circumstances to what her family experienced eighteen months ago. Construction begins next month, with the same local training model that has made the first well's maintenance record so strong.

We'll keep sharing updates as this second project develops, the same way we've shared Fatima's story with you today, because we believe you deserve to see exactly what your generosity built, not just in the moment of construction, but in the years that follow, when the real impact becomes visible in a girl's school attendance record and a mother's small garden.

A conversation with Amina, one year later

When our field coordinator visited Amina's family last month for the one year follow-up survey, she asked Amina what surprised her most about the past year. Amina didn't mention the water first. She mentioned the mornings, how strange it still felt, some days, to watch Fatima walk to school instead of toward the river, and how that small, ordinary sight had become the thing she was most grateful for. Water, she said, had always felt like the emergency. It took months for her to realize that what the well had actually given her family back was something quieter, time, and the ability to imagine a year ahead instead of just a day ahead.

That shift, from survival thinking to future thinking, is the part of this work that rarely fits into a single statistic, but it may be the most important outcome of all. Families with reliable water access consistently report, across every community we've worked in, that the change they value most isn't the water itself. It's what the water frees them to do with the rest of their day, their year, their children's future.

Why long-term follow-up matters to us

We don't consider a well finished at the ribbon-cutting. Every well we fund gets a one year, three year, and five year follow-up visit, both to verify the infrastructure is still functioning and to document what's changed for the families who depend on it. That follow-up is part of what your donation funds, not just the construction cost, but the years of accountability afterward that let us come back to you with real numbers instead of assumptions.

Thank you for making Fatima's ninety four percent possible. We can't wait to show you what the next well makes possible too.

See more stories like this, or fund the next well, at clearwaterinitiative.org/impact